MICHIGAN MEDICINE
UNIVERSITY OF MICHIGAN

HIV Pre- and Post-exposure Prophylaxis Guideline

HIV Pre-Exposure Prophylaxis

Clinical Setting	Therapy (not restricted to ID)	Required Lab testing
A) Sexually Active Adults and Adolescents (> 35 kg) who have had anal or vaginal sex in the past 6 months **AND any of the following:** 1) A sexual partner with HIV infection with detectable HIV viral load[1] 2) A recent bacterial STI 3) Inconsistent or no condom use with sexual partners B) **People who inject drugs AND** 1) Have had an injecting partner who is HIV positive AND/OR 2) Who share drug preparation or injection equipment with other people **If receiving HIV PrEP, STI PEP is also indicated with DoxyPEP (see guideline below)** [1]If partner is HIV infected with undetectable viral load on effective ART, then engage with shared decision making with patient regarding need for PrEP with risk/benefit discussion. **NOTE:** Both oral and injectable PrEP can be prescribed by any provider (not restricted to Infectious Diseases providers) **NOTE:** Adolescents under age 18 require parental consent for PrEP unless prescribed at a Title X clinic.	**Emtricitabine** 200 mg PO daily + **tenofovir disoproxil fumarate** 300 mg PO Daily (**TRUVADA**)[2] (requires CrCl > 60 mL/min) *OR* **Emtricitabine** 200 mg-**tenofovir alafenamide** 25 mg PO daily (**DESCOVY**)[2] (requires CrCl > 30 mL/min. If CrCl is < 30 consult ID) *OR* **Cabotegravir** (CAB) 600 mg gluteal IM injection (**APRETUDE**) at time 0, 4 weeks, then every 8 weeks.[3] *OR* **Lenacapavir** (LEN) 927 mg subcutaneous (SC) injection and 600 mg PO (**YEZTUGO**) on day 1, then 600 mg PO on day 2, then 927 mg SC every 26 weeks (6 months).[3] LEN has cytochrome P450 drug interactions. Evaluate potential interactions in ALL patients prior to initiation using the University of Liverpool checker. Supplemental LEN doses are necessary in patients receiving moderate-strong inducers of CYP3A4. Note that LEN CYP effects will persist for months after therapy discontinuation. *Provide adherence counseling for all patients who receive HIV PrEP regardless of formulation.* **NOTE:** CAB and LEN have long tails with gradually declining drug levels. High risk of resistant HIV infection if doses are missed. If drug is stopped, use oral PrEP x 12 months with follow up starting 2 months after last CAB dose and 6 months after last LEN dose. [2]Provide only a 90-day supply with 0 refills. Refill after 90 days only if required lab testing is performed. [3]Administered through injection clinics scheduled through MIST via therapy plan	Initial lab screening ALL patients: - HIV antigen/antibody - HIV quantitative PCR (within 7 days of initiation) - STI screening: oral, anal swabs and urine NAAT for gonorrhea and chlamydia (GC/CT), syphilis screen. Additional initial lab screening: *ORAL PrEP*: - Basic metabolic panel (BMP) - Hepatitis B serology[5] - Pregnancy screen - Lipid panel (Descovy only) Lab monitoring *ORAL PrEP*: *Every 3 months:* - HIV antigen/antibody - HIV 1 RNA PCR - NAAT for GC/CT based on anatomic site of exposure (e.g., rectal and/or pharyngeal swabs in addition to urine testing) - Syphilis screen - Pregnancy test *Every 6 months* – BMP if > 50 year old OR CrCl < 90 mL/min *Yearly* – BMP if < 50 year old AND CrCl > 90 mL/min – Lipid Panel (Descovy only) Lab monitoring *INJECTABLE PrEP (CAB and LEN)* *4 – 7 days prior to each injection[6]:* - HIV antigen/antibody - HIV 1 RNA PCR *Every 3-4 months:* - NAAT for GC/CT based on anatomic site of exposure (e.g., rectal and/or pharyngeal swabs in addition to urine testing) - Syphilis screen [5]Patients with active HBV infection should be referred to Infectious Diseases and Hepatology for HIV PrEP because of specific management considerations. [6]For ANY positive test, refer patient to ID clinic because of known challenges in diagnosing HIV infection with Cabotegravir failure. *Do not administer CAB or LEN if ANY test is positive*. Need more information? • **CDC HIV PrEP guidelines** • **National Clinician Consultation Center PrEP Line @ 1-855-448-7737** (9:00 AM – 8:00 PM EST)

ANTIMICROBIAL STEWARDSHIP
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HIV Non-occupational Post-Exposure Prophylaxis		
Clinical Setting	**Primary therapy**	**Comments**
Adults and adolescents ≥ 13 years old with CrCl > 60[6] within 72 hours of known or possible HIV exposure including: • Condom slippage or breakage • Lapse in condom use by serodiscordant or unknown status partners[6] • Other episodic exposure to blood, semen, vaginal secretions, or body fluids with visible blood contamination • Sexual assault • Needle sharing [6]Consult ID for further guidance if CrCl < 60 or if partner is known HIV positive with undetectable viral load	**Emtricitabine** 200 mg-**tenofovir disoproxil fumarate** 300 mg daily (**TRUVADA**) + **dolutegravir** 50 mg daily (**TIVICAY)** If combination product is not available, individual generic equivalents can be used	Duration: 28 days Baseline labs in exposed patient: - HIV antigen/antibody - site-specific NAAT testing for chlamydia/gonorrhea - RPR - Hep B surface antigen/antibody and core antibody - Hep C antibody - pregnancy test - comprehensive metabolic panel Follow up Testing: *4-6 weeks* - HIV Antigen/Antibody - RPR - Basic Metabolic Panel - Gonorrhea/Chlamydia NAAT if presumptive treatment not previously administered *3 months* - HIV Antigen/Antibody *6 months* - RPR - Hepatitis C antibody - Hepatitis B serology (if nonimmune at baseline) - HIV antigen/antibody (only if Hep C acquired in interim) Laboratory testing for source (if available) - HIV antigen/antibody test AND - HIV quantitative PCR if risk for acute HIV - Do NOT wait for results to start PEP, can discontinue if source testing is found to be negative. Exposures that do not warrant PEP: Kissing, spitting, oral-to-oral contact in the absence of mucosal damage (e.g., mouth-to-mouth resuscitation); human bites not involving blood Need more information? • Full CDC PEP prescribing information • **National Clinician Consultation Center PEP Line @ 1-888-448-4911** (9:00 AM – 8:00 PM EST)

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References	
	Citation

Revision History	
Date	**Revision**
7/2024	Added Cabotegravir
7/2025	Added Lenacapavir